

INFECTIOUS DISEASE CONSULTATION LETTER

Schuylkill Infectious Disease Associates – 833 Spruce St, Philadelphia PA

Re: Margaret Okafor DOB 03/22/1952 Consult date 04/14/2026

Setting: Inpatient consultation, hospital day 3 of admission for acute pyelonephritis (admitted 04/12/2026).

REASON FOR CONSULT: Persistent fever on empiric ceftriaxone; urine culture finalized today with ESBL-producing organism.

SUMMARY: 74F with recurrent UTIs, mechanical mitral valve on warfarin, T2DM, and CKD stage 3b (eGFR ~34). Urine culture 04/12: E. coli >100,000 CFU/mL, ESBL PRODUCER.

SUSCEPTIBILITIES:

Ceftriaxone	RESISTANT
Ciprofloxacin	RESISTANT
Levofloxacin	RESISTANT
TMP-SMX	RESISTANT (also SJS allergy - NEVER use)
Meropenem	SENSITIVE
Ertapenem	SENSITIVE
Nitrofurantoin	SENSITIVE (avoid: CrCl <40, pyelo)
Fosfomycin	SENSITIVE (cystitis only)

COURSE: Transitioned ceftriaxone to meropenem 1 g IV q12h (renal dosing) with defervescence within 24 hours. Blood cultures no growth.

RECOMMENDATIONS (page 2/2)

1. Complete 14-day total course: ertapenem 500 mg IV daily (renally dosed) x10 days via OPAT after discharge.
2. FOR FUTURE FEBRILE UTI EPISODES: recommend EMPIRIC CARBAPENEM (e.g., meropenem, renally dosed) PENDING CULTURES given documented ESBL history. Standard empiric ceftriaxone is expected to FAIL this organism.
3. Avoid TMP-SMX permanently (Stevens-Johnson syndrome).
4. Fluoroquinolones: resistant, and QTc 470 ms is a further caution.
5. Fever with a mechanical mitral prosthesis: maintain a low threshold to draw blood cultures BEFORE antibiotics and consider TEE if bacteremia or persistent fever (endocarditis evaluation).
6. Urology referral for recurrent UTI workup (placed).

Thank you for this consultation. Happy to re-consult on any recurrence.

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Schuylkill Infectious Disease Associates